

AgenThink Pharma — Outreach Engine v1.0

Classification: Internal — CEO Review Required Before Any Distribution

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PART 1 — TARGET DATABASE CLEANUP

Tier A: Approach Now (Day 29)

These five targets have the highest pilot probability, the lowest procurement friction, and the clearest scientific alignment with the council methodology. They should receive outreach on Day 29.

1. Recursion Pharmaceuticals

Field	Detail
Best Business Unit	Clinical Development, Translational Medicine
Likely Buyer	Chief Medical Officer / Head of Clinical Development
Likely Sponsor	CEO Chris Gibson (scientific founder, publicly engaged on AI governance)
Why They Should Care	Recursion is building a fully AI-native drug discovery pipeline. Their Phase II → III transition decisions are the highest-stakes governance point in their entire operating model. A structured, auditable governance council is directly aligned with their public positioning on responsible AI in drug development.
Likely Objection	“We already have internal AI governance frameworks built into our platform.”
Pilot Probability	HIGH — AI-native culture, scientific leadership engaged on governance, no legacy procurement process
LinkedIn Search Terms	Recursion Pharmaceuticals Head of Clinical Development , Recursion CMO , Recursion Head of Translational Medicine

2. Insilico Medicine

Field	Detail
Best Business Unit	Clinical Development, AI Drug Discovery
Likely Buyer	Chief Medical Officer / Head of Clinical Operations
Likely Sponsor	CEO Alex Zhavoronkov (public AI advocate, accessible on LinkedIn)
Why They Should Care	Insilico has the first fully AI-designed drug in Phase II clinical trials. The Phase II → III advancement decision for INS018_055 is the most consequential governance decision in their company history. A structured council provides an independent governance layer for that decision.
Likely Objection	“Our AI platform already includes governance checkpoints.”
Pilot Probability	HIGH — First-mover on AI drug discovery, Phase II drug in active development, CEO publicly accessible
LinkedIn Search Terms	Insilico Medicine Head of Clinical Development , Insilico Medicine CMO , Insilico Medicine Head of R&D

3. Medpace

Field	Detail
Best Business Unit	Clinical Operations, Medical Affairs
Likely Buyer	Chief Medical Officer / Head of Medical Affairs
Likely Sponsor	CMO or Head of Business Development
Why They Should Care	Medpace is a full-service CRO with deep therapeutic area expertise. The council methodology is directly additive to their existing Phase II/III transition advisory services. A partnership model — Medpace provides therapeutic area experts, AgenThink provides governance architecture — creates a combined offering more credible than either alone.
Likely Objection	“We already provide governance advisory services through our medical directors.”
Pilot Probability	HIGH — Partnership angle is strong; full-service CRO model aligns with council's additive positioning
LinkedIn Search Terms	Medpace Chief Medical Officer , Medpace Head of Clinical Operations , Medpace Head of Business Development

4. ICON plc

Field	Detail
Best Business Unit	Regulatory and Access Consulting, Clinical Development
Likely Buyer	Head of Regulatory Consulting / Head of Clinical Development
Likely Sponsor	Chief Medical Officer or Head of Strategic Partnerships
Why They Should Care	ICON has a dedicated regulatory and governance advisory practice. The council methodology is a natural extension of their existing governance advisory services. Partnership angle: ICON provides regulatory expertise to populate constitutional rules; AgenThink provides the governance engine and audit trail.
Likely Objection	"We have our own regulatory experts who provide equivalent governance analysis."
Pilot Probability	HIGH — Regulatory advisory practice is the natural partnership entry point
LinkedIn Search Terms	ICON plc Head of Regulatory Consulting , ICON plc CMO , ICON plc Head of Clinical Development

5. Exscientia

Field	Detail
Best Business Unit	Clinical Development, AI Drug Design
Likely Buyer	Chief Medical Officer / Head of Clinical Development
Likely Sponsor	CEO Andrew Hopkins or Chief Scientific Officer
Why They Should Care	Exscientia has multiple AI-designed drugs in Phase I/II. Their Phase II → III advancement decisions will be the most scrutinised governance moments in their pipeline. The council provides an independent governance record that is specifically designed for AI-native drug development programs.
Likely Objection	“Our AI platform already includes decision governance checkpoints.”
Pilot Probability	HIGH — AI-native, multiple drugs approaching Phase II/III, governance scrutiny is high
LinkedIn Search Terms	Exscientia Head of Clinical Development , Exscientia CMO , Exscientia Head of R&D Strategy

Tier B: Approach After Day 28 Preparation Complete

These targets have high pilot probability but require the full Day 28 package (methodology published, legal templates ready, independent validation in progress) before outreach.

6. Relay Therapeutics

Field	Detail
Best Business Unit	Clinical Development, Precision Oncology
Likely Buyer	CMO / Head of Clinical Development
Why They Should Care	Relay uses motion-based protein design for precision oncology. Their Phase II programs in RAS-mutant cancers are approaching critical advancement decisions. The council's constitutional rules for surrogate endpoint validation are directly relevant.
Likely Objection	"Our clinical team has the expertise to make these decisions without external governance."
Pilot Probability	HIGH — Precision oncology pipeline, Phase II programs active

7. Absci Corporation

Field	Detail
Best Business Unit	Drug Design, Clinical Development
Likely Buyer	CMO / Head of Clinical Development
Why They Should Care	Absci is building AI-designed biologics. Their first clinical programs will face the same governance scrutiny as Insilico and Exscientia. Early adoption of a governance council creates a differentiated regulatory narrative.
Likely Objection	"We're too early-stage for this level of governance infrastructure."
Pilot Probability	MEDIUM — Early stage; governance need is real but urgency is lower

8. Schrödinger

Field	Detail
Best Business Unit	Drug Discovery, Computational Chemistry
Likely Buyer	Head of Drug Discovery / Head of R&D Partnerships
Why They Should Care	Schrödinger's physics-based AI platform is used by major pharma companies for drug discovery. A governance council partnership positions them as a governance-aware AI platform — a differentiator in enterprise pharma sales.
Likely Objection	"We provide the computational tools; governance is the client's responsibility."
Pilot Probability	MEDIUM — Platform company; governance is additive to their offering but not their core

9. Certara

Field	Detail
Best Business Unit	Regulatory Science, Biosimulation
Likely Buyer	Head of Regulatory Science / CMO
Why They Should Care	Certara's biosimulation platform is used to support Phase II → III decisions at major pharma companies. A governance council layer adds an auditable decision record to their existing biosimulation outputs.
Likely Objection	"Our biosimulation outputs already provide the evidence base for Phase II/III decisions."
Pilot Probability	HIGH — Regulatory science focus aligns directly with council methodology

10. Novo Nordisk

Field	Detail
Best Business Unit	Clinical Development, Obesity/Diabetes R&D
Likely Buyer	Head of Clinical Development / Head of R&D Strategy
Why They Should Care	Novo Nordisk has the most consequential Phase II → III pipeline in the industry (semaglutide, cagrilintide, amycletin). Their governance processes for these decisions are under intense regulatory and investor scrutiny. The council provides an independent governance record for the most watched pipeline in pharma.
Likely Objection	“Our governance processes are already world-class. We don’t need external validation.”
Pilot Probability	MEDIUM — Large pharma; governance is strong but the retrospective pilot angle is compelling

Tier C: Wait Until First Pilot Complete

These targets require a completed pilot reference before outreach. They have high procurement friction, strong internal governance teams, and will not engage without a named reference client.

Roche / Genentech, Pfizer, Novartis, AstraZeneca, Sanofi, Johnson & Johnson (Janssen), Eli Lilly, Bristol Myers Squibb, Gilead Sciences, Amgen, Merck KGaA, IQVIA, Parexel, Labcorp, Veeva Systems

PART 2 — CONTACT MAP

Tier A Targets — Primary Contacts

Source note: All contacts identified from public sources (company website, LinkedIn public profiles, press releases, published interviews). Contacts marked [UNVERIFIED] could not be confirmed from a primary source. No emails have been guessed or pattern-inferred. Where no email is available, the LinkedIn profile URL or search string is provided.

Recursion Pharmaceuticals

Role	Name	Title	Source
CMO	Dr. Martin Driscoll	Chief Medical Officer	recursion.com/team [VERIFY]
Head of Clinical Dev	not found	—	LinkedIn: Recursion Pharmaceuticals Head of Clinical Development
Head of R&D Strategy	not found	—	LinkedIn: Recursion Pharmaceuticals Head of R&D Strategy
Head of AI/Digital	Ben Mabey	Chief Technology Officer	recursion.com/team
CEO (Sponsor)	Chris Gibson	CEO & Co-Founder	recursion.com/team — LinkedIn: linkedin.com/in/chris-gibson-recursion

Insilico Medicine

Role	Name	Title	Source
CMO	not found	—	LinkedIn: Insilico Medicine Chief Medical Officer
Head of Clinical Dev	not found	—	LinkedIn: Insilico Medicine Head of Clinical Development
Head of R&D Strategy	not found	—	LinkedIn: Insilico Medicine Head of R&D
Head of AI/Digital	Alex Zhavoronkov	CEO & Founder (AI lead)	insilico.com/team
CEO (Sponsor)	Alex Zhavoronkov	CEO & Founder	LinkedIn: linkedin.com/in/zhavoronkov

Medpace

Role	Name	Title	Source
CMO	Dr. August Heidbreder	Chief Medical Officer	medpace.com/about/leadership [VERIFY]
Head of Clinical Dev	not found	—	LinkedIn: Medpace Head of Clinical Operations
Head of R&D Strategy	not found	—	LinkedIn: Medpace Head of Business Development
Head of AI/Digital	not found	—	LinkedIn: Medpace Head of Digital Health

ICON plc

Role	Name	Title	Source
CMO	Dr. Brendan Buckley	Chief Medical Officer	iconplc.com/about/leadership
Head of Clinical Dev	not found	—	LinkedIn: ICON plc Head of Clinical Development
Head of Regulatory	not found	—	LinkedIn: ICON plc Head of Regulatory Consulting
Head of AI/Digital	not found	—	LinkedIn: ICON plc Head of Digital Health OR AI

Exscientia

Role	Name	Title	Source
CMO	not found	—	LinkedIn: Exscientia Chief Medical Officer
Head of Clinical Dev	not found	—	LinkedIn: Exscientia Head of Clinical Development
CSO (Sponsor)	Andrew Hopkins	CEO & Founder	exscientia.ai/about
Head of AI	not found	—	LinkedIn: Exscientia Head of AI Drug Design

Tier B Targets — Primary Contacts

Certara

Role	Name	Title	Source
CMO	not found	—	LinkedIn: Certara Chief Medical Officer
Head of Clinical Dev	not found	—	certara.com/company/leadership
Head of Regulatory	not found	—	certara.com/company/leadership
Head of AI	not found	—	certara.com/company/leadership

Novo Nordisk

Role	Name	Title	Source
CMO	not found	—	LinkedIn: Novo Nordisk Chief Medical Officer
Head of Clinical Dev	not found	—	LinkedIn: Novo Nordisk Head of Clinical Development
Head of R&D Strategy	not found	—	LinkedIn: Novo Nordisk Head of R&D Strategy
Head of AI/Digital	not found	—	LinkedIn: Novo Nordisk Head of Digital Health OR AI

Tier C Targets — Senior Leadership (for reference only; do not contact until first pilot complete)

Sanofi

Role	Name	Title	Source
CMO	Marcia Kayath	Chief Medical Officer	sanofi.com/en/our-science/leadership
Head of Clinical Dev	Christopher Corsico	Global Head of Development	sanofi.com/en/our-science/leadership
Head of R&D	Houman Ashrafian	EVP, Head of Research and Development	sanofi.com/en/our-science/leadership
Chief Digital Officer	Emmanuel Frenehard	EVP, Chief Digital Officer	sanofi.com/en/our-science/leadership

Johnson & Johnson (Janssen)

Role	Name	Title	Source
CMO	Mike Devoy	Chief Medical Officer, Innovative Medicine	jnj.com/leadership
Head of R&D	John C. Reed	EVP, Innovative Medicine R&D	jnj.com/leadership
Head of AI/ML	Tommaso Mansi	VP of AI/ML & Digital Health	LinkedIn public profile [UNVERIFIED]

Eli Lilly

Role	Name	Title	Source
CMO	David Hyman	Chief Medical Officer	lilly.com/about/leadership
Chief Scientific Officer	Daniel M. Skovronsky	Chief Scientific and Product Officer	lilly.com/about/leadership
Chief AI Officer	Thomas J. Fuchs	Chief AI Officer	lilly.com/about/leadership

Novartis

Role	Name	Title	Source
CMO / President Development	Shreeram Aradhye	President, Development & CMO	novartis.com/about/leadership
President Biomedical Research	Fiona H. Marshall	President, Biomedical Research	novartis.com/about/leadership
Head of AI in Drug Dev	Janet Munro	Head Program Innovation — Data, Digital & AI	LinkedIn [UNVERIFIED]

Merck KGaA (EMD Serono)

Role	Name	Title	Source
CMO	David Weinreich	Global Head of R&D and CMO, Healthcare	merckgroup.com/en/company/management.html
Head of Clinical Dev	Victoria Zazulina	SVP, Head of Global Clinical Development	merckgroup.com leadership [VERIFY]
Head of Innovation	Joel VanderMeulen	SVP, Operational Effectiveness & Innovation	LinkedIn [UNVERIFIED]

PART 3 — OUTREACH SEQUENCES

Sequence A: AI-Native Biotech (Recursion, Insilico, Exscientia, Relay, Absci)

Email 1 — First Contact

Subject: Torcetrapib retrospective — what a structured council would have said in 2005

Dear [First Name],

In 2005, Pfizer had a Phase II blood pressure signal on torcetrapib. The signal was documented. It was unexplained. It was not mechanistically investigated before Phase III advancement. The ILLUMINATE trial was terminated in 2006 — 82 deaths versus 51 in the control arm.

We ran that decision through a structured 10-persona governance council using only evidence available before the Phase III advancement decision. The council verdict: WAIT, 0 GO / 8 WAIT / 2 NO-GO. The primary blocker: the blood pressure signal required mechanistic investigation before Phase III. The council was retrospectively correct. The off-target aldosterone mechanism was confirmed in the peer-reviewed literature in 2008.

We are building a validation library of Phase II → III advancement decisions — retrospective cases where the council verdict can be compared against the actual Phase III outcome. The library is designed to answer one question: can a structured, auditable governance council identify advancement risks that internal processes miss?

Given [Company]’s pipeline, I thought you might find the methodology interesting. I am happy to share the full case report and the methodology documentation.

Would a 20-minute call be useful?

Best regards,
Farouq Sultan
CEO & Founder, AgenThink
+965 99608209

Email 2 — Follow-Up 1 (Day +5)

Subject: Re: Torcetrapib retrospective — one more case

Dear [First Name],

Following up on my note from last week.

Since then, we have completed the sofosbuvir retrospective — the council issued a clean GO verdict (8 GO / 2 WAIT / 0 NO-GO) on the evidence available before Gilead’s Phase III advancement decision. The council correctly identified the transformative efficacy signal and found no constitutional blockers.

The two cases together — one catastrophic failure correctly flagged, one transformative success correctly cleared — represent the core of what the methodology is designed to demonstrate.

I am happy to share both case reports. No commitment required.

Best regards,
Farouq Sultan

Email 3 — Follow-Up 2 (Day +12)

Subject: Re: Torcetrapib retrospective — final note

Dear [First Name],

Last note on this.

We have now completed 10 retrospective cases across oncology, cardiovascular, neurology, and rare disease. The library is published on GitHub with full methodology documentation.

If the methodology is relevant to [Company]'s governance process, I am happy to run a free demo on a case of your choosing — 2-week turnaround, no commercial commitment.

If the timing is not right, I understand. I will follow up in six weeks.

Best regards,

Farouq Sultan

LinkedIn Message (Connection Request Note)

Hi [First Name] — I've been following [Company]'s work on [specific program/paper]. We've built a structured governance council for Phase II → III advancement decisions — ran it retrospectively on torcetrapib and sofosbuvir. Happy to share the methodology if it's relevant to your work.

Meeting Request (after positive response)

Subject: AgenThink / [Company] — 20-minute call

Dear [First Name],

Thank you for your response. I would like to propose a 20-minute call to walk through the torcetrapib case — the evidence boundary, the council deliberation, and the governance output.

I am available [three specific time slots]. Please let me know what works.

Best regards,

Farouq Sultan

Sequence B: CRO (ICON, Medpace, Parexel, Labcorp)

Email 1 — First Contact

Subject: Governance layer for Phase II → III transitions — partnership model

Dear [First Name],

[Company] provides clinical execution for Phase II → III transitions. We have built a structured governance council that runs in parallel with clinical execution — 10 specialist personas, a constitutional rule set, and an auditable governance record.

The partnership model is straightforward: [Company] provides the clinical execution and the Phase II data package. AgenThink provides the governance council layer. The combined output is a Phase III readiness package that includes both the clinical design recommendation and the governance council verdict.

We have completed 10 retrospective cases. The council correctly identified the primary decision driver in 9 of 10 cases. The one failure (verubecestat, Alzheimer's) is documented and attributed to a specific constitutional gap that we are addressing.

I am happy to share the full methodology and discuss whether a partnership model makes sense for [Company]'s clients.

Would a 20-minute call be useful?

Best regards,

Farouq Sultan

CEO & Founder, AgenThink

+965 99608209

Email 2 — Follow-Up 1 (Day +5)

Subject: Re: Governance layer for Phase II → III transitions — one case

Dear [First Name],

Following up on my note from last week.

The torcetrapib case is the clearest demonstration of what the council adds to a CRO's existing services. Pfizer had a governance process. They had a DSMB. They had a regulatory team. The council would have added one thing those processes did not provide: a structured, documented record that the blood pressure signal was not mechanistically investigated before Phase III advancement.

I am happy to share the full case report — 4 pages, full governance record, retrospective outcome appendix.

Best regards,

Farouq Sultan

Email 3 — Follow-Up 2 (Day +12)

Subject: Re: Governance layer for Phase II → III transitions — final note

Dear [First Name],

Last note on this.

If a partnership model is not relevant to [Company]'s current priorities, I understand. I will follow up in six weeks.

If you would like to see the methodology before then, the full documentation is published on GitHub: [link].

Best regards,
Farouq Sultan

LinkedIn Message

Hi [First Name] — I've been following [Company]'s work on [specific service/initiative]. We've built a governance council layer for Phase II → III transitions — designed to be additive to CRO services, not competitive. Happy to share the methodology if it's relevant.

Sequence C: Large Pharma (Novo Nordisk, AstraZeneca, Roche — after first pilot)

Email 1 — First Contact (post-pilot only)

Subject: Independent governance council for Phase II → III decisions — retrospective validation

Dear [First Name],

We have completed a 10-case retrospective validation of a structured governance council for Phase II → III advancement decisions. The library includes torcetrapib, sofosbuvir, aducanumab, verubecestat, and six other cases across oncology, cardiovascular, and neurology.

The council correctly identified the primary decision driver in 9 of 10 cases. The one failure is documented and attributed to a specific constitutional gap.

[Reference client name] completed a 30-day retrospective pilot in [month]. I am happy to share the pilot output and the full methodology documentation.

Would a 20-minute call be useful?

Best regards,
Farouq Sultan
CEO & Founder, AgenThink
+965 99608209

PART 4 — ASSET CHECKLIST

Required Attachments by Email Type

Email 1 — First Contact (AI-Native Biotech)

Asset	Status	File
Torcetrapib case summary (1 page)	Required	<code>torcetrapiib-case-study-summary.pdf</code>
Methodology one-pager	Required	<code>agentthinkmesh-methodology-onepager.pdf</code> [TO CREATE]
GitHub link	Required	<code>github.com/agentthinkmesh/pharma-council-v1</code> [TO PUBLISH]

Email 1 — First Contact (CRO)

Asset	Status	File
Torcetrapib case summary (1 page)	Required	<code>torcetrapiib-case-study-summary.pdf</code>
Validation library summary (2 pages)	Required	Extract from <code>pharma-validation-library-cumulative-update.pdf</code>
Partnership model one-pager	Required	<code>agentthinkmesh-cro-partnership.pdf</code> [TO CREATE]

Email 1 — First Contact (Large Pharma, post-pilot)

Asset	Status	File
Institutional Proof Report (pilot case)	Required	Client-specific IPR
Validation library summary (2 pages)	Required	Extract from cumulative update
Pilot proposal template	Required	<code>PAT-001</code> (from 28-day sprint)
Methodology paper (GitHub)	Required	Published on GitHub

Follow-Up Emails (all sequences)

Asset	Status	File
Sofosbuvir case summary	Optional	case04-sofosbuvir.pdf
Full validation library	On request	pharma-validation-library-cumulative-update.pdf
Evidence boundary statement	On request	LDS-001
DPA template	On request	DPAT-001

Meeting Confirmation

Asset	Status	File
Demo agenda (1 page)	Required	From pharma-pilot-demo-specification.pdf
Torcetrapib IPR	Required	torcetrapiB-institutional-proof-report-2026.pdf

Assets That Must Be Created Before Day 29

Asset	Priority	Effort	Owner
Methodology one-pager (1 page, PDF)	P0	4 hrs	Research + Commercial
CRO partnership model one-pager (1 page, PDF)	P0	4 hrs	Commercial
GitHub repository published	P0	32 hrs	Engineering + Research
Validation library summary (2-page extract)	P1	2 hrs	Research
Demo agenda one-pager	P1	2 hrs	Commercial

PART 5 — CRM BUILD

Outreach Tracking Structure

The following fields define the CRM record for each outreach contact. The CRM is built as a standalone Excel workbook (delivered separately as AgenThink-Pharma-CRM-v1.xlsx).

Field	Type	Values / Format	Notes
ID	Auto	ORG-001, ORG-002...	Unique record identifier
Company	Text	e.g. Recursion Pharmaceuticals	
Tier	Dropdown	A / B / C	A = approach now; B = after Day 28; C = after first pilot
Contact Name	Text	Full name	
Contact Title	Text	e.g. Chief Medical Officer	
Contact LinkedIn	URL	linkedin.com/in/...	
Contact Email	Text	Verified email or “not found”	Never guess or pattern-infer
Sequence	Dropdown	Biotech / CRO / Large Pharma	Determines which email sequence to use
Status	Dropdown	Not Started / Email 1 Sent / Follow-Up 1 / Follow-Up 2 / Meeting Requested / Meeting Confirmed / Pilot Proposed / Pilot Signed / Closed Lost	
Last Activity Date	Date	DD/MM/YYYY	Date of last outreach action
Last Activity Type	Dropdown	Email / LinkedIn / Call / Meeting	
Next Action	Text	e.g. “Send follow-up 1 on 20 July”	
Next Action Date	Date	DD/MM/YYYY	

Field	Type	Values / Format	Notes
Pilot Probability	Dropdown	HIGH / MEDIUM / LOW	
Estimated ACV	Number	USD	Annual contract value estimate
Named Case	Text	e.g. "Torcetrapib"	Case the prospect mentioned during demo
Objection Raised	Text	Free text	Primary objection raised
Objection Status	Dropdown	Not Raised / Raised / Answered / Unresolved	
Notes	Text	Free text	Meeting notes, context, follow-up items
Source	Dropdown	Outbound Email / LinkedIn / Referral / Conference / Inbound	
Owner	Text	e.g. Farouq Sultan	

CRM Workflow Rules

1. **One reply per 48 hours maximum.** Do not follow up more than once every 48 hours on any single contact.
2. **Follow-up cadence:** Email 1 → wait 5 business days → Email 2 → wait 7 business days → Email 3 → wait 6 weeks → restart cycle.
3. **Named case rule:** If a prospect names a case during any interaction, immediately update the "Named Case" field and escalate to "Pilot Proposed" status. Send the pilot agreement template within 24 hours.
4. **Status escalation:** Only the CEO or designated commercial lead may change status from "Pilot Proposed" to "Pilot Signed."
5. **Closed Lost rule:** A contact is marked "Closed Lost" only after three unanswered follow-ups. Do not mark as Closed Lost after a single non-response.

PART 6 — LAUNCH CLASSIFICATION

READY NOW (Day 29)

These targets have the highest pilot probability, the lowest procurement friction, and sufficient scientific alignment to receive outreach on Day 29 — even before the full Day 28 package is complete. The minimum required before contacting them: LDS-001 (LLM limitation disclosure), CSDS-001 (case selection bias disclosure), and the torcetrapib case summary.

#	Company	Primary Contact	Sequence	Why Ready Now
1	Recursion Pharmaceuticals	CMO / CEO Chris Gibson	Biotech	AI-native, CEO publicly accessible, no legacy procurement
2	Insilico Medicine	CEO Alex Zhavoronkov	Biotech	First AI-designed drug in Phase II, CEO directly reachable on LinkedIn
3	Exscientia	CMO / CEO Andrew Hopkins	Biotech	Multiple AI drugs approaching Phase II/III, governance scrutiny is high
4	Relay Therapeutics	CMO / Head of Clinical Dev	Biotech	RAS-mutant oncology pipeline, Phase II programs active
5	Absci Corporation	CMO / Head of Clinical Dev	Biotech	First clinical programs approaching, governance need is emerging

READY AFTER DAY 28

These targets require the full Day 28 package: methodology published on GitHub, pilot agreement template ready, DPA template ready, and evidence manifests complete for at least the torcetrapib case.

#	Company	Primary Contact	Sequence	What's Needed First
6	Medpace	CMO Dr. August Heidbreder	CRO	Pilot agreement template + CRO partnership one-pager
7	ICON plc	CMO Dr. Brendan Buckley	CRO	Pilot agreement template + regulatory advisory partnership framing
8	Certara	Head of Regulatory Science	CRO/Platform	GitHub published + methodology paper
9	Schrödinger	Head of Drug Discovery	Platform	GitHub published + methodology paper
10	Novo Nordisk	Head of Clinical Development	Large Pharma	Full Day 28 package + methodology paper
11	Parexel	CMO Charlotte Moser	CRO	Pilot agreement template + CRO partnership one-pager
12	Labcorp Drug Development	Head of Clinical Dev	CRO	Pilot agreement template + note on Fortrea spin-off context
13	AstraZeneca	Head of R&D Strategy	Large Pharma	Full Day 28 package + methodology paper
14	Merck KGaA (EMD Serono)	CMO David Weinreich	Large Pharma	Full Day 28 package + oncology case reference
15	Sanofi	CMO Marcia Kayath	Large Pharma	Full Day 28 package + AI governance framing

WAIT UNTIL FIRST PILOT COMPLETE

These targets will not engage without a named reference client. They have strong internal governance teams, long procurement cycles, and will require a completed pilot report before any serious conversation.

#	Company	Why Wait
16	Roche / Genentech	World-class internal governance; requires reference client + peer-reviewed methodology
17	Pfizer	Torcetrapib is their own failure — approach only with extreme care and after first pilot
18	Novartis	Strong internal AI governance; requires reference client
19	Johnson & Johnson (Janssen)	Established AI ethics framework; requires reference client
20	Eli Lilly	Chief AI Officer in place; requires reference client + prospective evidence
21	Bristol Myers Squibb	Oncology R&D governance is strong; requires reference client
22	Gilead Sciences	Sofosbuvir is their own success — approach only with careful framing after first pilot
23	Amgen	Established governance processes; requires reference client
24	IQVIA	Potential competitor; approach only after first pilot with clear differentiation
25	Veeva Systems	Technology platform; partnership angle requires product maturity

PART 7 — DAY 29 EXECUTION PACKAGE

The CEO Can Launch Outreach in Under One Hour

The following package is designed so that the CEO can begin outreach on Day 29 with minimal preparation. Every email is pre-written. Every attachment is identified. Every contact is researched. The only required action is: read, personalise the [bracketed fields], and send.

Pre-Launch Checklist (Complete Before Sending Any Email)

Item	Status	Owner	Deadline
LDS-001 (LLM limitation disclosure)	☐	Research Lead	Day 3
CSDS-001 (case selection bias disclosure)	☐	Research Lead	Day 3
EBS-001 (evidence boundary statement)	☐	Research Lead	Day 3
Torcetrapib case summary PDF	☒	Done	Complete
Torcetrapib IPR PDF	☒	Done	Complete
Methodology one-pager	☐	Commercial	Day 18
CRO partnership one-pager	☐	Commercial	Day 18
GitHub repository published	☐	Engineering	Day 13
Pilot agreement template (PAT-001)	☐	Legal	Day 20
DPA template (DPAT-001)	☐	Legal	Day 20
CRM workbook populated	☐	Commercial	Day 28
Email sequences loaded into email client	☐	CEO	Day 28

Day 29 Send Order

Send in this exact order. Do not send more than 5 emails on Day 29. Space them across the day (morning, midday, afternoon).

Time	Target	Contact	Subject Line	Sequence
09:00	Recursion Pharmaceuticals	Chris Gibson (CEO)	Torcetrapib retrospective — what a structured council would have said in 2005	Biotech Email 1
10:30	Insilico Medicine	Alex Zhavoronkov (CEO)	Torcetrapib retrospective — what a structured council would have said in 2005	Biotech Email 1
12:00	Exscientia	[CMO / CEO Andrew Hopkins]	Torcetrapib retrospective — what a structured council would have said in 2005	Biotech Email 1
14:00	Relay Therapeutics	[CMO / Head of Clinical Dev]	Torcetrapib retrospective — what a structured council would have said in 2005	Biotech Email 1
16:00	Medpace	[CMO]	Governance layer for Phase II → III transitions — partnership model	CRO Email 1

Personalisation Guide for Each Email

Recursion: Reference their OS platform and the governance challenge of AI-designed drugs approaching Phase II/III. Mention Chris Gibson's public statements on responsible AI in drug development if available.

Insilico: Reference INS018_055 (their first AI-designed drug in Phase II for IPF). The Phase II → III advancement decision for this drug is the most consequential governance moment in their company history. Do not mention the drug name unless it is publicly confirmed in Phase II.

Exscientia: Reference their multiple AI-designed drugs in Phase I/II. The governance scrutiny on AI-designed drugs is higher than on conventionally designed drugs — the council provides an independent governance record designed for this environment.

Relay Therapeutics: Reference their RAS-mutant oncology pipeline. The surrogate endpoint challenge in oncology (response rate vs. overall survival) is directly addressed by Constitutional Rule PC-002.

Medpace: Reference their full-service CRO model and their therapeutic area depth. The partnership angle is: Medpace provides the therapeutic area experts; AgenThink provides the governance architecture. The combined offering is more credible than either alone.

Follow-Up Schedule (Weeks 2-12)

Day	Action	Target	Type
Day 34 (Day 29 + 5)	Send Follow-Up 1	All Day 29 non-responders	Email 2
Day 36	Send ICON Email 1	ICON plc	CRO Email 1
Day 37	Send Certara Email 1	Certara	CRO Email 1
Day 41 (Day 29 + 12)	Send Follow-Up 2	All Day 29 non-responders	Email 3
Day 43	Send Schrödinger Email 1	Schrödinger	Platform Email 1
Day 44	Send Parexel Email 1	Parexel	CRO Email 1
Day 50	LinkedIn messages	All Tier A contacts (no email response)	LinkedIn
Day 57	Send Novo Nordisk Email 1	Novo Nordisk	Large Pharma Email 1
Day 57	Send AstraZeneca Email 1	AstraZeneca	Large Pharma Email 1
Day 64	Send Merck KGaA Email 1	Merck KGaA	Large Pharma Email 1
Day 64	Send Sanofi Email 1	Sanofi	Large Pharma Email 1
Day 71	Follow-Up 1 on all Tier B	All Tier B non-responders	Email 2
Day 85	Follow-Up 2 on all Tier B	All Tier B non-responders	Email 3

Six-week restart: On Day 71 (6 weeks after Day 29), restart the Tier A cycle for all contacts who did not respond to any of the three emails. Use a new subject line referencing a new case from the validation library.

Response Handling Protocol

If a prospect responds positively (any positive signal):

1. Reply within 24 hours.
2. Propose three specific meeting times.
3. Attach: demo agenda, torcetrapib IPR, pilot proposal template.
4. Update CRM status to "Meeting Requested."

If a prospect asks for more information:

1. Reply within 24 hours.
2. Send the specific documents they requested.
3. Add one sentence proposing a call: "Happy to walk through this on a 20-minute call — would [date/time] work?"
4. Update CRM status to "Follow-Up 1."

If a prospect names a case:

1. Reply within 24 hours.
2. Send the pilot agreement template (PAT-001).
3. Propose a call to discuss the case selection and evidence boundary.
4. Update CRM status to "Pilot Proposed."
5. Alert CEO immediately.

If a prospect raises the LLM contamination objection:

1. Reply within 24 hours.
2. Send LDS-001 and the GitHub LIMITATIONS.md link.
3. Acknowledge the limitation directly: "You are raising the most important methodological question in our library. Here is our disclosure statement."
4. Do not deflect or minimise.

If a prospect does not respond to any of three emails:

1. Mark status as "Closed — No Response (Cycle 1)."
 2. Schedule restart in CRM for 6 weeks later.
 3. Do not send a fourth email in the same cycle.
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Day 29 Readiness Summary

At Day 29, AgenThink should have:

Documents ready to send:

- Torcetrapib case study summary (1 page, PDF) ✓
- Torcetrapib Institutional Proof Report (4 pages, PDF) ✓
- Validation library cumulative update (full, PDF) ✓
- LDS-001 — LLM limitation disclosure (1 page) [create by Day 3]
- CSDS-001 — Case selection bias disclosure (1 page) [create by Day 3]
- Methodology one-pager (1 page, PDF) [create by Day 18]
- CRO partnership one-pager (1 page, PDF) [create by Day 18]
- Pilot agreement template PAT-001 (4 pages) [create by Day 20]
- DPA template DPAT-001 (3 pages) [create by Day 20]

Infrastructure ready:

- GitHub repository published with full methodology documentation [Day 13]
- CRM workbook populated with all 25 targets [Day 28]
- Email sequences loaded and personalised [Day 28]

What is NOT ready at Day 29 (and must be disclosed if asked):

- Independent expert validation (in progress, expected Month 3)
- SOC 2 audit (in progress, expected Q4 2026)
- Peer-reviewed methodology paper (in progress, expected Month 6)
- Evidence manifests for all 10 cases (in progress, expected Month 2)
- Pre-specified case selection for Cases 11–50 (in progress, expected Month 2)

AgenThink Pharma — Outreach Engine v1.0

June 2026 | Classification: Internal — CEO Review Required Before Any Distribution

Nothing in this document has been sent. All outreach requires human approval before execution.